

PATIENT HISTORY:

PRE-OP DIAGNOSIS: Bladder cancer.

POST-OP DIAGNOSIS: Same.

PROCEDURE: Radical cystectomy, creation of ileal conduit, left nephroureterectomy.

FINAL DIAGNOSIS:**PART 1:**

LYMPH NODES, RIGHT PELVIC, EXCISION -

NO EVIDENCE OF MALIGNANCY IN TWO (2) LYMPH NODES EXAMINED.

PART 2:

LYMPH NODES, LEFT PELVIC, EXCISION -

NO EVIDENCE OF MALIGNANCY IN TWO (2) LYMPH NODES EXAMINED.

PART 3:

URETER, RIGHT, DISTAL MARGIN -

BENIGN URETER WITH REACTIVE UROTHELIAL CHANGES AND NO EVIDENCE OF NEOPLASIA.

PART 4:

URINARY BLADDER AND URETHRA, RADICAL CYSTECTOMY -

A. INVASIVE HIGH GRADE UROTHELIAL CARCINOMA, 4.5 CM, (WHOMISUP) WITH EXTENSIVE NECROSIS. CARCINOMA EXTENSIVELY INVOLVES THE LEFT BLADDER WALL AND LEFT TRIGONE, WITH ENCASEMENT OF THE DISTAL LEFT URETER AND PRE-STENOTIC LEFT URETERAL DILATATION. THE CARCINOMA INVADERS THE DETRUSOR MUSCLE AND EXTENDS INTO THE PERIVESICAL FAT.

B. INVASIVE UROTHELIAL CARCINOMA IS PRESENT IN THE SUBMUCOSAL TISSUE AT THE PERIURETHRAL MARGIN. THE FOCUS OF CARCINOMA AT THIS MARGIN MEASURES APPROXIMATELY 0.7 CM (Slide 4A). ALL OTHER FINAL SURGICAL MARGINS ARE FREE OF NEOPLASIA.

C. FOCAL UROTHELIAL CARCINOMA IN SITU IS IDENTIFIED (Slide 4G).

D. NO PERINEURAL INVASION IS IDENTIFIED.

E. ANGIOLYMPHATIC INVASION IS IDENTIFIED, WITH TUMOR THROMBI PRESENT IN LYMPHATICS AND ALSO IN A SMALL VEIN (Slides 4C, 4E).

F. NON-NEOPLASTIC BLADDER WITH CHRONIC CYSTITIS AND SMALL URACHAL REMNANT IN BLADDER DOME.

G. TNM PATHOLOGIC STAGING = T3a N0 MX.

H. TNM HISTOPATHOLOGIC GRADE = G3 (see microscopic description).

PART 5:

KIDNEY AND URETER, LEFT, NEPHROURETERECTOMY -

A. BENIGN KIDNEY WITH SEVERE CHRONIC PYELONEPHRITIS, ASSOCIATED WITH SEVERE GLOBAL AND SEGMENTAL GLOMERULOSCLEROSIS, CHRONIC VASCULAR AND TUBULOINTERSTITIAL DISEASE, AND HYDRONEPHROSIS.

B. URETER WITH FOCAL UROTHELIAL ATYPIA OF UNDETERMINED SIGNIFICANCE (DYSPLASIA VERSUS SEVERE REACTIVE ATYPIA; Slide 5B).

C. NO INVASIVE CARCINOMA IDENTIFIED.

COMMENT:

The case was discussed with Dr.

SYNOPTIC PRIMARY URINARY BLADDER/URETHRA TUMORS

- A. Location of primary tumor: 5, 6, 8
1. Dome 4. Right wall 7. Ureter 10. No residual tumor
2. Anterior wall 5. Left wall 8. Urethra
3. Posterior wall 6. Trigone 9. Multicentric
- B. Procedure: 2
1. Partial cystectomy 3. Cystoprostatectomy
2. Total cystectomy 4. Other
- C. Size (if multicentric, maximum dimension of the most deeply invasive or largest tumor): 4.5 cm
C1: Tumor configuration: 4
1. Papillary 4. Ulcerated.
2. Solid/nodular 5. Cystic
3. Flat
- D. Non-invasive neoplastic lesions:
1. Papilloma 4. Flat CIS
2. Inverted papilloma 5. Other
3. Papillary TCCa
- E. Invasive neoplasia: 1
1. Urothelial (Transitional cell) carcinoma 7. Small cell/neuroendocrine carcinoma
2. Urothelial carcinoma with squamous differentiation 8. Mixed urothelial carcinoma
3. Urothelial carcinoma with glandular differentiation 9. Lymphoma/leukemia
4. Squamous carcinoma 10. Sarcoma
5. Adenocarcinoma 11. Metastasis/direct extension from a contiguous primary
6. Sarcomatoid carcinoma 12. Other
- F. Grade: 3-4 (Ash for TCCa grades 1-4) / 3 (WHO for TCCa grades 1-3)/HG (WHO / ISUP (LMP, LG, HG))
- G. Non-neoplastic and other conditions:
1. Von Brunn's nests, cystitis cystica/glandularis, intestinal metaplasia
2. Nephrogenic metaplasia/adenoma
3. Granulomatous cystitis
4. Interstitial cystitis
5. Inflammatory pseudotumors
6. Caruncle
7. Melakoplakia
8. Other
- H. Margins of resection: 1, Periurethral
1. Positive
2. Negative

UUID: C35A6F61-D5C4-4F8C-B82B-1349E6CDA43
TCGA-BT-A20P-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: [Signature]	Date Reviewed: 9/11/11	

I. Angiolymphatic invasion: 1
 J. Perineural invasion: 2 1. Yes 2. No
 K. Mitotic activity: 2 per 10 high power fields 2. No
 L. Flat dysplasia in background: 1 1. Yes 2. No
 M. Number of positive lymph nodes: 0
 N. Total number of lymph nodes examined: 4
 O. Extracapsular spread of lymph node metastases: n/a 1. Yes 2. No
 Prostate/seminal vesicles (if cystoprostatectomy)
 P1. Involved by bladder neoplasm: # 1. Yes 2. No
 P2. Concurrent primary invasive prostatic neoplasm: # 1. Yes 2. No
 P3. High grade PIN: # 1. Yes 2. No
 Q. Additional findings: #
 1. Urothelial dysplasia 2. Therapy associated changes
 R. TNM stage: T 3a N 0 M X